

FIBROMYALGIA

- Consider fibromyalgia if the patient has had general body pain above and below the waist, affecting both sides of the body for more than 3 months associated with at least 11 of 18 tender points (see picture) on palpation.
- Fibromyalgia diagnosis more likely if any of: woman, family history, fatigue, reduced ability to think and remember clearly, mood or sleep disturbances.
- Check for other causes of general body pain:
 - If weight loss → 23.
 - Screen for a joint problem: patient to place hands behind head; then behind back. Bury nails in palm and open hand. Press palms together with elbows lifted. Walk. Sit and stand up with arms folded. If unable to do screen comfortably → 62.
 - Check CRP, Hb, TSH and test for HIV → 110.
- **A doctor must make or confirm the diagnosis of fibromyalgia.** If joint problem, HIV positive, blood results abnormal or uncertain, consider another diagnosis and refer.



Press tender points with the pressure that would blanch a fingernail. Compare with a control site on forehead.

Assess the patient with fibromyalgia

Assess	When to assess	Note
Symptoms	Every visit	• Manage symptoms as on symptom pages. Ask patient to identify the 3 symptoms that bother her/him most and focus on these. • Do not dismiss all symptoms as fibromyalgia: exclude treatable and serious illness. If unsure, refer.
Chronic pain	At diagnosis	Further assess and advise on chronic pain → 61.
Sleep	Every visit	If patient has difficulty sleeping → 87.
Stressors	Every visit	Help identify psychosocial stressors that may exacerbate symptoms. If stress or anxiety → 86.
Depression	Every visit	In the past month, has patient: 1) felt down, depressed, hopeless or 2) felt little interest or pleasure in doing things? If yes to either → 143.
Chronic arthritis	Every visit	If patient also has chronic arthritis, give routine care 151.

Advise the patient with fibromyalgia

- The cause is unknown but may be a result of generalised hypersensitivity of the nervous system, so patient feels more pain than others, despite normal muscles and joints.
- The patient may also have irritable bowel syndrome, tension-headache, chronic fatigue syndrome, interstitial cystitis, sleep disturbances or depression.
- Explain that s/he will have good days and bad days, but that fibromyalgia does not get worse over time. It is not life-threatening, but there is no cure.
 - Advise the patient against overuse of painkillers (e.g. paracetamol and ibuprofen) as they are often not helpful for fibromyalgia and may have unwanted side effects.
 - Advise patient to keep as active as possible: start with 5 minutes of gentle walking every day and build up by 1 minute a day until able to walk or run for 30 minutes at least 3 times per week.
 - Encourage good sleep habits → 87.
 - Refer to available support group and helpline → 178.
 - If no better with a combination of education, exercise and medication, refer for cognitive behavioural therapy if available.

Treat the patient with fibromyalgia

- Give **paracetamol** 1g 4-6 hourly (up to 4g in 24 hours) as needed.
- If no better with education and exercise, give **amitriptyline**¹ 10mg at bedtime. Increase by 5mg every 2 weeks until improvement (maximum dose 75mg).
- If still symptomatic after 3 months on maximum dose, refer/discuss.

A supportive relationship with the same health practitioner can contain frequent visits for multiple problems. Review patient 6 monthly once stable.

¹Avoid if on bedaquiline.